**This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.**

Patient Group Direction

# for the administration of seasonal influenza vaccines (IIVc, aIIV, IIVr and IIVe) for protection against seasonal influenza, 2026/27 season

Patient Group Direction, version 007, issued 30 September 2026. Supersedes Version 006, 14 September 2026.

Owner and authorising organisation: Get Real Health Limited, company number 12744898, registered office Zenvite Health Unit 23, St. Asaph Business Park, St. Asaph, Denbighshire, LL17 0LJ. This Patient Group Direction is authorised on behalf of Get Real Health Limited by its Medical Director and its Head Pharmacist, whose signatures for this version are on the authorisation page at the end of the document. It applies to the registered healthcare professionals named under it by a pharmacy or clinic that has adopted it through a Get Real Health service agreement, at the premises stated in that adoption, and to no other organisation.

## Guideline Summary: seasonal influenza vaccination, 2026/27

**The season**

- All influenza vaccines marketed in the UK for the 2026 to 2027 season are trivalent. The B/Yamagata lineage has been removed, as it has not been detected globally since March 2020.
- WHO recommends for the 2026 to 2027 northern hemisphere season, for egg-based vaccines: an A/Missouri/11/2025 (H1N1)pdm09-like virus, an A/Darwin/1454/2025 (H3N2)-like virus and a B/Tokyo/EIS13-175/2025 (B/Victoria lineage)-like virus.
- For cell culture, recombinant protein and nucleic acid based vaccines: an A/Missouri/11/2025 (H1N1)pdm09-like virus, an A/Darwin/1415/2025 (H3N2)-like virus and a B/Pennsylvania/14/2025 (B/Victoria lineage)-like virus.
- Influenza vaccination is given annually before the season begins. Protection develops over about 10 to 14 days.
- The NHS programme opens for appointments from 17 August 2026. This PGD covers privately funded vaccination and does not replace the national programme.

**Choosing the right vaccine**

- The vaccine must be licensed for the age of the individual. Under this PGD, cell-based vaccine (IIVc) is the only vaccine that may be given to anyone under 18 years.
- Adjuvanted vaccine (aIIV) is for individuals aged 50 years and over and is the preferred option for older adults, in whom the adjuvant improves the immune response.
- Recombinant vaccine (IIVr) and egg-cultured vaccine (IIVe) are for adults, with IIVe restricted to 18 to 64 years under this PGD because from 65 years the adjuvanted, recombinant or cell-based vaccine is used instead, in line with Green Book chapter 19 and JCVI advice for older adults.
- Egg allergy: IIVc and IIVr are egg-free and contain no ovalbumin. These are the appropriate choices for individuals with egg allergy, including those with a history of anaphylaxis to egg. aIIV and IIVe are egg-cultured and contain ovalbumin.
- This PGD covers injectable inactivated vaccines only. It does not cover live attenuated influenza vaccine (Fluenz nasal spray), which is the vaccine of choice for children under the NHS programme.

**Children**

- Children aged 2 to 17 years may only receive IIVc under this PGD, as it is the only listed vaccine licensed below 18 years.
- A child under 9 years who is receiving influenza vaccine for the first time should receive 2 doses at least 4 weeks apart, in accordance with Green Book chapter 19 and the SPC. Book the second dose at the first appointment.
- Consent must be obtained from a person with parental responsibility, or from the young person where they are assessed as Gillick competent.
- Children eligible under the NHS childhood programme should be advised that they can be vaccinated free of charge, usually with the nasal spray vaccine.

**Co-administration**

- Inactivated influenza vaccine may be given at the same visit as COVID-19 vaccine and other vaccines, at separate sites, preferably in different limbs.
- Where given in the same limb, sites should be at least 2.5 cm apart. The site of each vaccine must be recorded.

# Patient Group Direction

for the administration of

**Seasonal influenza vaccines: IIVc, aIIV, IIVr and IIVe**

for protection against seasonal influenza, 2026/27 season

## Healthcare professionals covered and training

| | Healthcare professionals allowed to work under this PGD and requirements |
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| Qualifications and Registration | ● Pharmacist registered and practicing with the GPhC<br>● Pharmacy technician registered and practicing with the GPhC |
| Training and assessment | Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team.<br>● All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines<br>● All users must have completed immunisation training in line with the National Minimum Standards and Core Curriculum for Immunisation Training, updated for the current season<br>● All users must be competent in the recognition and management of anaphylaxis, and in vaccine handling and cold chain management<br>● All users vaccinating children must be competent in the assessment of consent in children and young people, including Gillick competence, and in vaccinating children<br>● All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required<br>● All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so.<br>● Must work in compliance with the SOPs of their own employer<br>● All users must have appropriate indemnity in place to cover this service |
| Further training and responsibilities | ● All users must be up to date with CPD requirements and appraisal<br>● All users must be up to date with MHRA and safety alerts with regards to medical products<br>● All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005 |

## Clinical condition or situation to which this PGD applies

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| PGD Indication | Active immunisation against seasonal influenza for the 2026/27 season using inactivated trivalent influenza vaccine, in accordance with the current Summary of Product Characteristics for the product supplied and Chapter 19 of Immunisation Against Infectious Disease (the Green Book). Cell-based vaccine (IIVc) may be given from 2 years of age; adjuvanted vaccine (aIIV) from 50 years; recombinant vaccine (IIVr) from 18 years; and egg-cultured vaccine (IIVe) from 18 to 64 years. This PGD covers privately funded vaccination, whether or not the individual is eligible under the NHS programme. |
| Inclusion criteria | ● Aged 2 years and over, and within the age range of the specific vaccine to be administered as set out in the vaccines table below.<br>● Requires influenza vaccination for the 2026/27 season and either does not qualify for NHS vaccination, or qualifies but prefers to be vaccinated privately.<br>● Has not already received an influenza vaccine for the 2026/27 season, other than a child under 9 years attending for the second of two doses.<br>● Valid informed consent obtained, from a person with parental responsibility where the individual is under 16 and not Gillick competent.<br>● No contraindication to the vaccine as set out below. |
| Exclusion criteria | ● Aged under 2 years. Refer to the NHS childhood programme, the GP or a service commissioned to vaccinate this age group.<br>● Outside the licensed or PGD age range for every vaccine held in stock. In particular, no vaccine other than IIVc may be given under this PGD to anyone under 18 years.<br>● Known hypersensitivity to the active substances, or to any excipient or residue listed in the current SPC for the product to be given.<br>● Confirmed anaphylactic reaction to a previous dose of any influenza vaccine.<br>● Egg allergy where only an egg-cultured vaccine (aIIV or IIVe) is available. Use IIVc or IIVr, which are egg-free, or refer.<br>● Acute severe febrile illness. Postpone until recovered. A minor infection without fever is not a contraindication.<br>● Has already received an influenza vaccine for the 2026/27 season, other than a child under 9 years attending for the second of two doses.<br>● Bleeding disorder where intramuscular injection has not been assessed as safe by a clinician familiar with the individual's bleeding risk.<br>● Valid informed consent not obtained, or no person with parental responsibility available to consent for a child under 16 who is not Gillick competent. |
| Cautions including any relevant action to be taken | ● Facilities and trained staff for the management of anaphylaxis must be available, with immediate access to adrenaline (epinephrine) 1 in 1,000 injection and a telephone.<br>● Observe every individual for 15 minutes after vaccination, seated, and record that the observation period was completed. Syncope can occur, particularly in adolescents; ensure procedures are in place to avoid injury from faints.<br>● Anxiety related reactions, including vasovagal reactions |

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| | (syncope), hyperventilation or stress related reactions (for example dizziness, palpitations, increases in heart rate, alterations in blood pressure, paraesthesia, hypoaesthesia and sweating) may occur in association with the vaccination process itself. Stress related reactions are temporary and resolve on their own. Individuals should be advised to bring symptoms to the attention of the vaccination provider for evaluation. It is important that precautions are in place to avoid injury from fainting.<br>● Children under 9 years receiving influenza vaccine for the first time require 2 doses at least 4 weeks apart. Book the second dose at the first appointment and record that this is dose 1 of 2.<br>● Egg allergy is not a barrier to vaccination provided an egg-free vaccine is used. IIVc and IIVr contain no ovalbumin. aIIV and IIVe are egg-cultured and contain ovalbumin at or below 1 microgram per dose.<br>● Pregnancy and breastfeeding: inactivated influenza vaccine is recommended at any stage of pregnancy and is safe while breastfeeding. Pregnant women are eligible under the NHS programme and should be advised of this.<br>● Immunosuppression: the immune response may be reduced. Vaccination is still recommended and the individual should be advised that protection may be limited.<br>● Anticoagulation and bleeding disorders: individuals on stable anticoagulation, including those on warfarin who are up to date with INR testing and whose latest INR is below the upper threshold of their therapeutic range, may be vaccinated intramuscularly with a 23 gauge or finer needle, with firm pressure applied without rubbing for at least 2 minutes. Advise on the risk of haematoma.<br>● Where the individual or child is eligible for NHS vaccination, they must be told that they can receive the vaccine free of charge on the NHS before proceeding with a private supply. |
| Actions if patient is excluded or declines treatment | Advise on alternative options and how these can be accessed, including their GP practice and, where they are eligible, the NHS programme. Where an egg-free vaccine is needed and none is in stock, arrange supply or refer rather than administering an egg-cultured vaccine. Explain the benefit of vaccination and the risks of influenza and its complications. Document any advice given and the decision reached. Inform or refer to the GP as appropriate. |

## Vaccines covered by this PGD

*Confirm the presentation, licensed age range and strain statement against the current SPC and the pack before administration.*

| Vaccine | Age under this PGD | Dose and route | Notes |
|---|---|---|---|
| Cell-based Trivalent Influenza Vaccine Seqirus (IIVc), inactivated, surface antigen, cell cultured | 2 years and over | 0.5 ml intramuscular | Egg-free, no ovalbumin. The only vaccine in this PGD that may be given under 18 years. Licensed from 6 months, restricted to 2 years and over under this PGD. Children under 9 receiving influenza vaccine for the first time need 2 doses at least 4 weeks apart. |
| Adjuvanted Trivalent | 50 years and | 0.5 ml intramuscular | Preferred for older adults. Egg-cultured, |

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| Influenza Vaccine Seqirus (aIIV), inactivated, surface antigen, MF59C.1 adjuvanted, egg cultured | over | | ovalbumin at or below 1 microgram per 0.5 ml dose. Not suitable where egg allergy requires an egg-free vaccine. |
| Supemtek TIVr (IIVr), inactivated, recombinant, cell cultured | 18 years and over | 0.5 ml intramuscular | Egg-free, contains no ovalbumin. Suitable for adults with egg allergy. |
| Egg-cultured inactivated vaccine (IIVe), for example Vaxigrip or Influenza vaccine TIV MYL | 18 to 64 years | 0.5 ml intramuscular | Egg-cultured, ovalbumin at or below 0.05 micrograms (Vaxigrip) or 0.1 micrograms (TIV MYL) per 0.5 ml dose. Not suitable where egg allergy requires an egg-free vaccine. Licensed from 6 months, restricted to 18 to 64 years under this PGD. |

## Details of the medicines

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| Strains for the 2026/27 season | As stated in the current SPC for the product supplied. Egg-based vaccines (aIIV and IIVe): an A/Missouri/11/2025 (H1N1)pdm09-like virus, an A/Darwin/1454/2025 (H3N2)-like virus and a B/Tokyo/EIS13-175/2025 (B/Victoria lineage)-like virus. Cell culture and recombinant vaccines (IIVc and IIVr): an A/Missouri/11/2025 (H1N1)pdm09-like virus, an A/Darwin/1415/2025 (H3N2)-like virus and a B/Pennsylvania/14/2025 (B/Victoria lineage)-like virus. Confirm the strain statement on the pack before use. |
| Legal category | POM |
| Black triangle | Several of these products carry black triangle status. Check the current SPC and MHRA guidance for the product supplied, and report all suspected adverse reactions via the Yellow Card scheme. |
| Off-label use | This PGD does not authorise any off-label use. Administration must be within the licensed age and dose stated in the current SPC for the product supplied. Where this PGD sets a narrower age range than the licence, the PGD range applies. |
| Storage | Store at +2°C to +8°C. Do not freeze; discard if the vaccine has been frozen. Store in the original packaging to protect from light. Vaccine exposed to conditions outside this range must be quarantined and risk assessed in accordance with UKHSA Vaccine Incident Guidance before any decision on continued use. |
| Route/method of administration | ● Administer by intramuscular injection into the deltoid muscle of the upper arm. In younger children where deltoid muscle bulk is insufficient, use the anterolateral thigh.<br>● Shake or invert the pre-filled syringe as directed in the SPC before use. Inspect visually and do not administer if the appearance differs from that described in the SPC.<br>● Where given with other vaccines, including COVID-19 vaccine, use separate sites, preferably different limbs, or at least 2.5 cm apart in the same limb. Record the site of each.<br>● Where the intramuscular route is not suitable, refer rather than administering by an alternative route under this PGD. |
| Dose and frequency of administration | A single 0.5 ml dose for all vaccines and ages covered by this PGD. One dose per individual per season, except that a child under 9 years receiving influenza vaccine for the first time requires 2 doses at least 4 weeks apart. Revaccination is required each influenza season. |
| Quantity to be administered and/or supplied | One 0.5 ml dose per administration. |

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| Maximum or minimum treatment period | One dose for the 2026/27 season, or two doses at least 4 weeks apart for a child under 9 years being vaccinated for the first time. This PGD expires at the end of the 2026/27 season and must be reviewed and reissued for the following season. |
| Disposal | Dispose of used syringes, needles and any discharged vaccine in a UN-approved puncture-resistant sharps container in accordance with local arrangements and HTM 07-01. |
| Drug interactions | May be given at the same time as other vaccines at a separate site. The immune response may be reduced in individuals receiving immunosuppressive treatment; this is not a reason to withhold vaccination. Refer to the current SPC for the product supplied. |
| Adverse effects | Very common and common: pain, redness, swelling or induration at the injection site, headache, fatigue, myalgia, malaise and low grade fever. In children, irritability, drowsiness and loss of appetite are also common. Local and systemic reactions may be more frequent with adjuvanted vaccine. Uncommon and rare: paraesthesia, generalised skin reactions and hypersensitivity reactions including anaphylaxis. Refer to the current SPC and PIL for the full list. |
| Yellow card reporting | Report suspected adverse reactions to the MHRA via the Yellow Card scheme at https://yellowcard.mhra.gov.uk. Document any adverse reaction in the individual's record and inform their GP. |
| Records to be kept | Record the following information:<br>● that valid informed consent was given, and by whom where the individual is a child: where the individual is under 16, the name and relationship of the person with parental responsibility, or the basis of the Gillick competence assessment<br>● name of individual, address, date of birth and GP with whom the individual is registered<br>● name of immuniser<br>● name and brand of vaccine, and the vaccine type (IIVc, aIIV, IIVr or IIVe)<br>● date of administration<br>● dose, form and route of administration<br>● quantity administered<br>● batch number and expiry date<br>● anatomical site of vaccination<br>● the season for which the vaccine was given (2026/27), and for children under 9 whether this is dose 1 or dose 2<br>● advice given, including advice given if excluded or declines treatment<br>● details of any adverse drug reactions and actions taken<br>● that the vaccine was administered via PGD<br>Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. The individual's GP should be informed.<br>● Adults aged 18 years and over: keep records for 8 years<br>● Children aged under 18 years: keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday) |

## Patient information

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| Written information to be given to | Offer the marketing authorisation holder's patient information leaflet |

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| patient or carer | (PIL) provided with the vaccine, and a written record of the vaccine given with the date, brand, vaccine type and batch number. For a child under 9 receiving the first of two doses, give written confirmation of the date the second dose is due. |
| Follow-up advice to be given to patient or carer | Explain that protection develops over about 10 to 14 days and lasts for the season, and that revaccination is needed each year. Inform the individual, parent or carer of possible side effects and their management, and advise them to seek medical advice in the event of an adverse reaction and to report it via the Yellow Card scheme. Advise that the vaccine cannot cause influenza and does not protect against other respiratory infections, and to seek advice if they become unwell. Vaccines do not provide 100% protection. |

## Key references

- Immunisation Against Infectious Disease (the Green Book), Chapter 19 (Influenza)
- UKHSA: all influenza vaccines marketed in the UK for the 2026 to 2027 season, February 2026, including age indications and ovalbumin content
- JCVI statement on influenza vaccines for the 2026 to 2027 season
- WHO recommended composition of influenza virus vaccines for use in the 2026 to 2027 northern hemisphere influenza season, 27 February 2026
- Summary of Product Characteristics for each vaccine administered, current version
- UKHSA Vaccine Incident Guidance
- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions
- British National Formulary (BNF) and BNF for Children

## Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

| Name and address of pharmacy premises / clinic premises to which this PGD relates | |
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| Name of healthcare professional | Job title | Registration number | Signature |
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| Valid from date | Expiry date |
|---|---|
| 30/9/26 | 31/7/27 |

## PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or vaccine.

This section must be signed by the superintendent or clinical lead responsible for this service. To be completed by the adopting pharmacy. These are the adopting organisation's own signatories and must not be Get Real Health staff.

*To be completed by the adopting pharmacy: the first block is signed by that pharmacy's superintendent or clinical lead, and the second by its professional group signatory. Get Real Health signs the authorship block below, not these.*

| Name of superintendent / clinical lead | Job title & name of organisation | Signature | Date |
|---|---|---|---|
| | | | |
| Name of professional group signatory | Job title & name of organisation | Signature | Date |
| | | | |

## Change history to version 003

Later versions are recorded in the Version and change record at the end of this document.

| Version number | Change details | Date |
|---|---|---|
| 001 | Development and issue of new PGD, adjuvanted trivalent influenza vaccine (Fluad), 65 years and over | 1st October 2025 |
| 002 | Reissued for the 2026/27 season as a multi-vaccine PGD. Cell-based vaccine (IIVc) added as the primary vaccine from 2 years of age, replacing Fluad as the default. Adjuvanted vaccine (aIIV) retained for 50 years and over, recombinant vaccine (IIVr) added for 18 years and over, and egg-cultured vaccine (IIVe) added for 18 to 64 years, so that any stock available can be used. Strain statements added for both egg-based and cell culture or recombinant vaccines. Egg allergy handling added, with IIVc and IIVr identified as egg-free. Paediatric provisions added: IIVc only under 18, the two dose schedule for children under 9 being vaccinated for the first time, parental consent and Gillick competence, thigh administration in young children, and paediatric record retention. Live attenuated nasal vaccine (Fluenz) explicitly outside this PGD. | 30th July 2026 |
| 003 | Amendments from clinical review by C. Pilkington: anxiety related reactions added to cautions, and patient advice now states that vaccines do not provide 100% protection. Adoption and authorisation blocks corrected: those signatures belong to the adopting pharmacy, not to Get Real Health. | 6th August 2026 |
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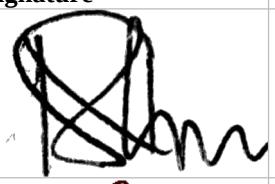
**Version and change record**
Version: v007
Issued: 30 September 2026
Supersedes: Version 006, 14 September 2026
Valid from: 30 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Training before first use, applied to every Get Real Health PGD on 30 September 2026: the staff section now states that each practitioner should complete the Get Real Health training for this PGD, or equivalent training elsewhere, before first working under it, must read the PGD in full before first use, and that the date of completion is recorded. Training is recorded; it is not a condition of access, and the employing organisation remains responsible for being satisfied of competence. No clinical content changes.

**Previous version records**
Version: v006
Issued: 14 September 2026
Supersedes: Version 005, 11 September 2026
Valid from: 14 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Identity and training, applied to every Get Real Health PGD on 14 September 2026 after two adopting pharmacies wrote: the Get Real Health logo is in the page header; the organisation that owns and authorises this direction, Get Real Health Limited, is named under the strapline with its company number and registered office; the requirement that practitioners must previously have used a PGD is replaced by a requirement for PGD training and a competence assessment, so that a newly registered pharmacist or technician who has completed the training may work under it; and the earlier change history table is headed with the versions it covers.
Previous version records
Version: v005
Issued: 11 September 2026
Supersedes: Version 004, 11 September 2026
Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Guidance summary now says why IIVe is limited to 18 to 64 years under this PGD (an adjuvanted, recombinant or cell-based vaccine is used from 65).
Observation after vaccination is now 15 minutes seated for every individual, as in the other vaccine PGDs, rather than only where there is a history of allergy.
Records row now requires the name and relationship of the person with parental responsibility, or the basis of the Gillick assessment, for an individual under 16.
Wording corrections from the tool alignment and adversarial review of the consultation tools, 11 September 2026. Each correction resolves a contradiction inside the document or a plain error, taking the safer reading; no indication is widened, no dose raised and no exclusion removed. Questions that need a clinical decision are recorded separately and are not changed here.
Previous version records
Version: v004
Issued: 11 September 2026
Supersedes: Version 003, 6 August 2026
Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Structural clean-up applied to every live Get Real Health PGD on 11 September 2026, following the adversarial review of 10 September: the adopting pharmacy's authorisation block is blank, for the pharmacy's own signatories; one signed authorisation page for this version replaces the earlier signature blocks; every validity cell states this version's dates (valid from 11 September 2026, expiry 31 July 2027); narration about earlier versions, the consultation tool and individual customers is removed from the body of the document; the change records of earlier reissues are carried below as previous version records. No clinical criterion changes unless listed.


GET REAL
HEALTH

## Signed on behalf of Get Real Health

Version v007, 30 September 2026.

| Name | Qualification | Signature | Date |
|---|---|---|---|
| Nitin Shori | Medical Director<br>GMC: 6047293 |  | 30 September 2026 |
| Chris Pilkington | Head Pharmacist<br>GPhC: 2046322 |  | 30 September 2026 |

*Both authorising signatories reviewed this version together on 30 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs. This version is not valid without both signatures.*